

Dear Dr. Balcarras,

I am writing to resubmit the manuscript “Healthcare Analytics Challenges: A Three-Pillar Framework Connecting Analytics Maturity, Workforce Agility, and Technical Enablement” (ms#96541) as a *Viewpoint* article, revised in response to Decision D (major revision and re-review).

Response to Decision D

I am grateful to you and to the two external reviewers for the constructive and detailed assessment. The reviews materially strengthened the manuscript, and I have addressed every editorial and reviewer comment. A full point-by-point response accompanies this resubmission; the summary below highlights the principal changes.

Summary of Revisions

Engagement with recent literature and evidence (Editor; Reviewer Q3):

- Added empirical evidence that human oversight measurably improves AI-generated analytics, including a prospective study in which human-verified large-language-model output exceeded a human-only workflow, and a text-to-SQL validation gate that improved semantic correctness.
- Added database query-log evidence on query reuse and diversity to address the scope and practicality of the framework.
- In total, 14 references spanning 1994-2026 were added, the majority from 2016-2026.

Conceptual clarity (Reviewer T1):

- Added explicit, cited definitions of the three title constructs (analytics maturity, workforce agility, and technical enablement) at the head of each evidence pillar, grounded in the established literature.

Methodological caveats (Reviewer Q1, Q4):

- Added a self-selection-bias caveat to the analytics-maturity evidence, grounded in the organizational-survey methodology literature.
- Added a Limitations paragraph acknowledging that the framework complements, rather than replaces, investment in workforce training and analytics literacy.

Currency and consolidation (Reviewer Q6; Reviewer T2, T4):

- Replaced aging workforce statistics with contemporary (2021-2025) evidence; the single remaining two-decade-old reference is now framed explicitly as a historical benchmark.
- Consolidated duplicated turnover statistics to a single mention.

Figure revision (Reviewer T3):

- Revised Figure 1 to add a feedback edge from Organizational Memory to the Knowledge Base, depicting continuous learning rather than a static knowledge base.

Formatting and compliance (Editor):

- Converted all in-text citations to numbered square brackets.
- Removed heading numbering and the italic sub-table titles that stood in place of subheadings.

- Added a Funding statement distinct from the Acknowledgments, and a generative-AI disclosure within the manuscript.
- Completed all reference details (volume, issue, pages, DOI) and the Abbreviations list, with acronyms defined at first use.

The revised manuscript remains a *Viewpoint* of 4,532 body words (within the 5,000-word limit) with an unstructured abstract of 304 words.

Article Type

This submission is a *Viewpoint* article: an evidence-based scholarly perspective on mitigating institutional amnesia in healthcare analytics, grounded in peer-reviewed literature and knowledge management theory. As the Viewpoint article type does not permit IMRD section headings, descriptive thematic headers are used throughout, as detailed in the point-by-point response.

Thank you for your continued consideration.

Respectfully,

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